

## ASCII MASTER FLOW – PANEL 1/12: Coverage cube

POPULATION -> breadth: who is covered, all citizens or identified beneficiaries

SERVICES -> depth: promotive, preventive, curative and rehabilitative care

FINANCIAL PROTECTION -> height: how little the household pays at the point of care

RULE -> a system can widen one edge while narrowing another; test all three

MUST REMEMBER: Universal health coverage requires promotive, preventive, curative,...

## ASCII MASTER FLOW – PANEL 2/12: Policy and mission card

POLICY 2017 -> raise public health expenditure to 2.5 per cent of gross domestic product

ITS STATUS -> a policy document, not an enacted statute; commitments, not entitlements

RURAL MISSION -> launched 12 April 2005 for the rural population and vulnerable groups

URBAN MISSION -> Union Cabinet decision dated 1 May 2013 created it as a sub-mission

SPECIAL FOCUS -> Empowered Action Group States, North East, Jammu and Kashmir, Himachal

BENCHMARK -> facilities measured against the Indian Public Health Standards

## ASCII MASTER FLOW – PANEL 3/12: Two pillars kept apart

PILLAR ONE -> wellness centres and health mandirs; comprehensive primary health care

ITS PACKAGE -> twelve services including non-communicable disease and mental health

ITS STAFF -> community health officer or mid-level health provider

PILLAR TWO -> cashless secondary and tertiary hospitalisation cover

ITS QUANTUM -> up to five lakh rupees per family per year at empanelled hospitals

## ASCII MASTER FLOW – PANEL 4/12: Two enrolment routes

CORE ROUTE -> eligibility-based, identified through census deprivation criteria

AGE ROUTE -> every citizen aged seventy and above may enrol regardless of income

PRECISION -> the age route is universal only for that age group

ERROR TO AVOID -> writing an age-specific expansion up as universal cover for all ages

## ASCII MASTER FLOW – PANEL 5/12: Source discipline card

HEALTH AUTHORITY -> implements the hospitalisation pillar; empanels and pays claims

HEALTH ACCOUNTS -> official estimates of public, private and out-of-pocket spending

SEQUENCE -> 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22

RULE -> never confuse the publication date with the accounting year

MEANING -> a high share signals weak public provision, not high utilisation

## ASCII MASTER FLOW – PANEL 6/12: Impoverishment mechanism

ILLNESS -> care is sought from a provider outside the funded public layer

-> OUT-OF-POCKET PAYMENT at the point of service

-> CATASTROPHIC SPENDING: above a threshold share of household income or consumption

-> ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation

LINK -> this is where the health owner meets the poverty owner

CLOSE DISTINCTION: UHC is not free hospitalisation alone, insurance coverage is not...

## ASCH MASTER FLOW – PANEL 7/12: Referral spine

SUB-CENTRE -> first contact; outreach and community-level services

PRIMARY HEALTH CENTRE -> preventive, promotive and basic curative care

COMMUNITY HEALTH CENTRE -> specialist outpatient and inpatient care

DISTRICT HOSPITAL -> secondary care and the gateway to tertiary referral

USE -> name the failing tier; a vacancy, a diagnostic gap and a specialist shortage differ

## ASCII MASTER FLOW – PANEL 8/12: Marketisation and its harms

PRIVATE DOMINANCE IN CURATIVE CARE -> often unregulated or under-regulated

-> HIGH COSTS and unnecessary procedures

-> CATASTROPHIC SPENDING and inequitable access

BALANCE -> do not deny private capacity, and do not assume regulation is automatic



## ASCLII MASTER FLOW – PANEL 9/12: Regulatory toolkit

CLINICAL ESTABLISHMENTS ACT, 2010 -> registration and regulation of establishments

ITS CONDITION -> States must adopt it or notify equivalent legislation; adoption varies

PRICE CAPPING -> exercised over devices by the pharmaceutical pricing authority

TREATMENT GUIDELINES -> promote rational care and cost control; uptake varies

RULE -> name the adoption gap a regulatory recommendation is meant to close

## ASCH MASTER FLOW – PANEL 10/12: Financing families and chain

SOCIAL HEALTH INSURANCE -> contribution-linked pooling, as in the employees system

TAX-FUNDED -> general revenue, as in the health mission and the hospitalisation pillar

CHAIN -> revenue collection to risk pooling to strategic purchasing to provision

GAP -> insurance leaves prevention, medicines and outpatient spending unresolved

TRAP -> calling the tax-funded pillar a contributory insurance scheme

## ASCII MASTER FLOW – PANEL 11/12: Community health mapping

EQUITY -> reach the underserved first rather than the easiest to reach

PARTICIPATION -> village health, sanitation and nutrition committees

INTER-SECTORAL ACTION -> water, sanitation, nutrition and gender equality

APPROPRIATE TECHNOLOGY -> frontline health activists and auxiliary nurse midwives

INSTITUTION -> the upgraded wellness centre as the named Indian delivery point

## ASCII MASTER FLOW – PANEL 12/12: Capacity limits and answer spine

CONDITION -> Union financing and standards depend on State delivery capacity

GAP -> workforce vacancies and uneven capacity limit identical national schemes

MENTAL HEALTH -> a statutory right and a tele-service still need district professionals

OPEN -> name the coverage dimension tested; BUILD -> provision plus regulation

CLOSE -> graded verdict with a dashboard-count caveat and a reversal condition

EVIDENCE LIMIT: AUTHORITY / IMPLEMENTATION: Map Union-state-local responsibilities,...